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Adrabetadex Provider Net Cost Calculator Concept Brief

Prepared for the Beren Therapeutics Launch Team

Audience: Internal medical, commercial, market access, reimbursement, and finance stakeholders

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1. Opening Position

The Adrabetadex Provider Net Cost Calculator helps Beren evaluate whether treatment centers can administer Adrabetadex under real-world reimbursement conditions.

Adrabetadex may have a compelling clinical profile, but provider adoption will still depend on whether centers can acquire the drug, administer it, bill correctly, receive payment, and remain financially whole.

The calculator translates payer rules, site-of-care requirements, channel decisions, coding status, and manufacturer support into a clear provider-level financial result.

Provider Question	Why It Matters
Who buys the drug?	Determines whether the provider carries inventory and cash-flow risk.
Who bills for the drug?	Determines whether the provider can recover drug cost or only bill for administration.
What does the provider pay?	Acquisition cost may differ from WAC depending on 340B status, discounting, or channel.
What does the payer reimburse?	Expected payment under Medicare, Medicaid, commercial, or contracted scenarios.
How long does payment take?	Estimated days to payment and working-capital exposure.
What if the claim is denied?	Denial exposure, appeal recovery, and rework burden.
What closes the gap?	The support needed to make a thin or negative scenario financially workable.

Aetna Example for Non-Technical Readers

Aetna's policy for Brineura, a rare-disease therapy administered into the central nervous system, shows why a calculator like this is necessary. The policy does not simply ask whether the product is clinically appropriate. It requires precertification, diagnosis by or in consultation with a specialist in metabolic disease or lysosomal storage disorders, confirmed diagnosis by enzyme assay or genetic testing, and administration by or under a physician knowledgeable in intraventricular administration. It also ties reimbursement to covered coding, including applicable HCPCS and procedure codes where criteria are met.

The practical point is simple: for provider-administered rare-disease therapies, payer rules, specialist requirements, coding, and site capability affect whether the claim can be paid. The calculator must capture those mechanics, not just product price.

Source note: Aetna Clinical Policy Bulletin 0442, Lysosomal Storage Disorder Treatments.

2. Core Thesis

The calculator converts payer rules, site-of-care dynamics, and channel mechanics into treatment-center economics.

It is not a list-price model. It is not a manufacturer gross-to-net model. It is not a clinical value model.

In plain English, WAC helps size the starting exposure. It does not show what the provider pays, gets reimbursed, or carries while waiting for payment.

It is a provider-facing reimbursement execution model that estimates:

Model Output	Meaning for a Non-Technical Audience
Expected reimbursement	What the provider expects to be paid.
Inventory at risk	How much drug cost the provider is carrying before payment.
Time to cash	How long the provider waits to be paid.
Denial exposure	How much money is at risk if the claim is denied or delayed.
Administration economics	Whether the provider is paid enough for the work of giving the drug.
Break-even support	The support needed so the provider does not lose money.

3. Why the Calculator Is Needed

A provider can believe in the clinical value of a therapy and still hesitate to administer it if the financial pathway is unclear.

For Adrabetadex, the calculator is needed because several reimbursement issues can change whether the provider is financially whole.

Launch Friction	Simple Explanation	Why It Matters
Buy-and-bill	The provider buys the drug, gives it, bills the payer, and waits for payment.	Creates inventory and cash-flow risk.
White-bagging	A specialty pharmacy sends the drug to the provider for administration.	Reduces provider inventory risk but removes drug margin.
340B acquisition	Eligible hospitals may acquire the drug at a lower price.	Can make hospital economics stronger than non-340B settings.
Medicaid claim integrity	Medicaid may require correct NDC, coding, and state-specific billing details.	Coverage can exist while payment still fails.
HCPCS timing	Product-specific billing codes may not be available at launch.	Interim coding can create claim uncertainty.
Hospital contracting	Hospitals may need contracting clarity before they stock or administer the drug.	Internal hospital economics can shape adoption.
PBM / specialty pharmacy rules	Payers may require use of a specific specialty pharmacy.	Determines whether the provider can buy-and-bill.
Manufacturer support	Support may be needed to keep the provider whole.	Shows the minimum support needed to close the provider gap.

4. How the Calculator Works

The calculator uses a simple product and payment flow.

Buy-and-Bill Pathway	White-Bagging Pathway
Manufacturer -> Hospital / COE / Provider -> Provider acquires drug -> Provider administers drug -> Provider bills payer -> Provider waits for payment -> Provider net recovery or shortfall	Manufacturer -> Specialty pharmacy -> Specialty pharmacy supplies drug -> Provider administers only -> Provider bills administration -> Provider administration economics only

The calculator applies one formula across all scenarios:

Provider net position = drug reimbursement + administration revenue + provider-facing support - drug acquisition cost - labor and pharmacy handling - billing and denial rework - financing cost from delayed payment		
Step	What the Calculator Does	Plain-English Output
1. Starts with gross drug exposure	Uses WAC and dosing frequency.	How large the drug cost is.
2. Applies acquisition basis	Uses WAC, 340B, discount, rebate, or white-bagging.	What the provider actually has at risk.
3. Applies reimbursement basis	Uses payer, Medicare, Medicaid, contract, or manual payment input.	What the provider expects to be paid.
4. Adds administration costs	Adds labor, pharmacy handling, billing, overhead, and related costs.	What it costs to provide the service.
5. Adds claim friction	Adds denial risk, appeal recovery, rework, and days to payment.	How payment delays or denials affect the provider.
6. Tests support levels	Adds provider-facing support where needed.	The support needed to avoid a provider loss.
7. Produces a result	Calculates net recovery or shortfall.	Workable, thin, negative, or site-specific.

5. What the Calculator Will Show

The calculator should make the economics visible to people who do not normally think in reimbursement terms.

Question	Calculator Output	Why the Audience Should Care
Can the provider afford to start treatment?	Inventory exposure and expected reimbursement.	Shows whether a site may hesitate to stock or administer.
Does reimbursement cover drug cost?	Drug spread before operating cost.	Shows whether payment covers acquisition.
Does the provider remain whole after all costs?	Net recovery after labor, handling, denial, and timing costs.	Shows the true provider result.
Which sites are most viable?	COE, HOPD, physician office, Medicaid, and white-bagging comparison.	Helps prioritize launch sites.
Which payer rules create risk?	Prior authorization, specialist, coding, sourcing, and payment flags.	Shows where payer friction affects uptake.
What support closes the gap?	Break-even manufacturer support.	Shows what action is needed to make a scenario workable.
How does a comparator perform?	Same economics applied to a relevant therapy.	Shows whether Adrabetadex is better, worse, or similar under real reimbursement conditions.

Client-Facing Close

The purpose of the calculator is to make provider economics understandable.

Adrabetadex may be clinically compelling, but launch success still depends on whether treatment centers can buy the drug, administer it, bill correctly, get paid, and avoid carrying unacceptable financial risk.

The calculator shows where the economics work, where they are thin, where they fail, and what action is needed to improve the provider pathway.

Source Notes

Source notes are included to support the model framing without overloading the brief with technical citations.

Source Category	Purpose
Beren internal Adrabetadex assumptions	Supports WAC, dosing, and working model assumptions.
Aetna CPB 0442	Supports payer-rule example for rare-disease CNS-administered therapy.
CMS Part B payment limit / initial-period WAC rule	Supports initial reimbursement benchmark concept.
CMS HCPCS Level II process	Supports product-specific coding timing.
HRSA 340B ceiling price / new drug estimation	Supports 340B acquisition scenario logic.
Medicaid PAD / EPSDT resources	Supports Medicaid claim-integrity and pediatric coverage framing.